

# Cuba Information Repository

*Compiled May 2026*

## 1. CUBA / US RELATIONS

### Early History

#### Spanish-American War and Early U.S. Control (1898–1902)

Spain relinquished Cuba at the end of the Spanish-American War in 1898. The U.S. occupied the island until 1902, when it granted Cuba independence, but with strings attached: America reserved the right to intervene in Cuban affairs and secured a permanent lease on a naval base at Guantánamo Bay.

De Guzman, C., & Jeyaretnam, M. (2026, March 17). The crisis in Cuba, explained. *Time*.  
<https://time.com/article/2026/03/17/cuba-economic-energy-crisis-trump-us-explainer/>

## 2. TIMELINE: Cuba / US Relations (1959–2026)

Primary source for timeline entries unless otherwise noted: Council on Foreign Relations.

### 1959 — Castro Takes Power

Fidel Castro and a group of guerrilla fighters overthrow U.S.-backed President Fulgencio Batista and establish a socialist state.

### 1960 — The Mallory Memo (April 6)

Deputy Assistant Secretary of State for Inter-American Affairs Lester Mallory wrote a classified memo foundational to U.S. Cuba policy which stated that "*every possible means should be undertaken...to bring about hunger, desperation and overthrow of government.*"

CFR Editors. (2026, May 21). *U.S.-Cuba relations*. Council on Foreign Relations.  
<https://www.cfr.org/articles/us-cuba-relations>

Mallory, L. D. (1960, April 6). \*The decline and fall of Castro\* [Declassified memorandum]. U.S. Department of State. National Security Archive, George Washington University.  
<https://nsarchive.gwu.edu/document/27400-document-1-state-department-memorandum-decline-and-fall-castro-secret-april-6-1960>

National Security Archive. (2022, February 2). Cuba embargoed: U.S. trade sanctions turn sixty. *National Security Archive*.  
<https://nsarchive.gwu.edu/briefing-book/cuba/2022-02-02/cuba-embargoed-us-trade-sanctions-turn-sixty>

### 1960: Eisenhower Restricts Trade and Cuts Ties

After Cuba built closer ties with the Soviet Union and nationalized U.S.-owned property, President Dwight D. Eisenhower restricted trade and cut diplomatic relations.

### 1961: Bay of Pigs and the Foreign Assistance Act

President John F. Kennedy backed a CIA-supported invasion by Cuban exiles. The Cuban government defeated the group within days. That same year, Congress passed the Foreign Assistance Act, which expanded foreign aid for some countries and restricted it for others, including Cuba.

### **1962: Full Embargo Imposed**

The Kennedy administration imposed a full embargo, cutting off nearly all trade with Cuba.

### **1962: The Cuban Missile Crisis**

A thirteen-day standoff began after the U.S. discovered Soviet nuclear missiles in Cuba. It ended publicly when Kennedy agreed not to invade Cuba, and secretly agreed to remove U.S. missiles from Turkey.

### **1966: Cuban Adjustment Act**

The U.S. made it easier for Cubans to immigrate, giving many a path to residency after arriving.

### **1977: Limited Diplomatic Communication Restored**

The U.S. and Cuba reopened limited diplomatic communication.

### **1980: The Mariel Boatlift**

Approximately 125,000 Cubans left for the U.S. after Castro allowed people to depart.

### **1982: Cuba Designated a State Sponsor of Terrorism**

The U.S., under the Reagan Administration, placed Cuba on its state sponsor of terrorism list.

### **1985: Radio Martí Launched**

The U.S. launched Radio Martí to broadcast into Cuba. The Cuban government viewed it as propaganda.

### **1992: Cuban Democracy Act**

President George H.W. Bush signed the Cuban Democracy Act, tightening the embargo and restricting trade, including limiting ships that traveled to Cuba from entering U.S. ports.

### **1994–1995: Migration Agreements and "Wet Foot, Dry Foot"**

The U.S. and Cuba reached migration agreements including the "wet foot, dry foot" policy: Cubans intercepted at sea were returned to Cuba, while those who made it to U.S. shores could apply for permanent residency. President Obama ended this policy in January of 2017, during the final days of his presidency.

### **1996: Helms-Burton Act**

After Cuba shot down two civilian planes, the U.S. passed the Helms-Burton Act, locking the embargo into law and making it significantly harder to lift through future administrations.

### **1998: Cuban Intelligence Agents Arrested**

Five Cuban intelligence agents were arrested in the U.S. on espionage charges.

**1999: Elián González**

A custody battle over Elián González, a Cuban boy rescued at sea after his mother drowned attempting to flee Cuba, became a major political conflict between the U.S. and Cuba.

**2000: Cuba-Venezuela Alliance**

Cuba strengthened ties with Venezuela, exchanging services like healthcare workers for discounted oil.

**2008: Raúl Castro Took Over**

Fidel Castro stepped down and transferred power to his brother Raúl Castro.

**2009: Obama Eased Some Restrictions**

President Barack Obama eased restrictions on travel and remittances to Cuba.

**2009: U.S. Aid Worker Arrested**

A U.S. aid worker was arrested in Cuba, creating new diplomatic tensions.

**2011–2013: Cuba's Economic Reforms**

Cuba introduced economic reforms allowing more private business activity.

**2014: Diplomatic Relations Restored**

The U.S. and Cuba announced they would restore diplomatic relations after more than 50 years.

**2015: Cuba Removed from Terrorism List; Embassies Reopen**

The U.S. removed Cuba from its terrorism list and both countries reopened embassies.

**2016: Obama Visited Cuba**

In March, Obama visited Cuba and expanded travel and business opportunities between the two countries.

**2016: Fidel Castro Died**

Fidel Castro died in late November at age 90.

**2017: Trump Reinstated Restrictions**

President Donald Trump reinstated restrictions on travel and business with Cuba.

**2017: Embassy Staffing Cut After Health Incidents**

The U.S. cut embassy staff in Havana after unexplained health incidents affected diplomatic personnel.

**2018: Miguel Díaz-Canel Became President**

Raúl Castro stepped down as president and Miguel Díaz-Canel took over.

**2019: Helms-Burton Activated; Cruises Banned**

The Trump administration activated parts of the Helms-Burton Act and further restricted travel, including banning cruise ships from stopping in Cuba.

### **2021: Cuba Redesignated as State Sponsor of Terrorism**

In January of 2021, the Trump administration redesignated Cuba as a state sponsor of terrorism.

### **2021: End of the Castro Era**

Raúl Castro stepped down from his final leadership role, ending more than six decades of Castro family rule.

### **2021: Mass Protests and Government Crackdown**

Large protests broke out across Cuba over worsening economic conditions. The government responded with a crackdown.

### **2022: Biden Eased Some Restrictions**

The Biden administration eased some restrictions, including on travel and remittances.

### **2023: Deportation Flights Resumed**

The U.S. resumed deportation flights to Cuba as migration from the island increased.

### **2025: Cuba Briefly Removed from, Then Returned to, Terrorism List**

The Biden Administration removed Cuba from the Terrorism List on January 14, 2025, and Trump reinstated Cuba to the list on January 20, 2025.

### **2025: Trump Expanded Sanctions**

The Trump administration expanded sanctions and further limited travel and financial activity between the U.S. and Cuba.

### **January 29, 2026: Executive Order: Embargo Intensifies Restrictions**

Trump signed Executive Order 14380, declaring Cuba an "unusual and extraordinary threat" to U.S. national security. Oil deliveries are blocked, resulting in blackouts up to 20 hours a day.

Executive Order No. 14380, 91 Fed. Reg. 8987 (Feb. 3, 2026).

<https://www.whitehouse.gov/presidential-actions/2026/01/addressing-threats-to-the-united-states-by-the-government-of-cuba/>

### **May 1, 2026: Second Executive Order: Expanded Secondary Sanctions**

President Trump froze U.S. assets of any foreign individual or entity operating in key sectors of the Cuban economy: energy, defense, metals and mining, financial services, and security. The order forced foreign banks to choose between access to Cuba and access to the U.S. financial system.

The White House. (2026, May 1). *Imposing sanctions on those responsible for repression in Cuba and for threats to United States national security and foreign policy* (Executive Order No. 14404). *Federal Register*, 91 FR 25061.

<https://www.federalregister.gov/documents/2026/05/07/2026-09173/imposing-sanctions-on-those-responsible-for-repression-in-cuba-and-for-threats-to-united-states>

White House Fact Sheet:

<https://www.whitehouse.gov/fact-sheets/2026/05/fact-sheet-president-donald-j-trump-imposes-sanctions-on-cuban-regime-officials-responsible-for-repression-and-threats-to-u-s-national-security-and-foreign-policy/>

### **May 7, 2026: UN Experts Condemned Blockade as "Energy Starvation"**

A joint statement issued by three UN human rights special rapporteurs - Surya Deva, Sofia Monsalve Suarez, and Pedro Arrojo-Agudo - condemned the U.S. fuel blockade as "energy starvation." The statement noted that 187 nations have voted every year at the UN General Assembly to end the U.S. embargo. Only the U.S. and Israel dissent.

Source: Office of the United Nations High Commissioner for Human Rights. (2026, May 7). United States must end energy starvation of Cuba: UN experts. OHCHR.

<https://www.ohchr.org/en/press-releases/2026/05/united-states-must-end-energy-starvation-cuba-severe-human-rights-impacts-un>

### **May 20, 2026: United States Indicted Raúl Castro**

The United States unsealed an indictment against Raúl Castro and five Castro regime co-defendants for the 1996 shoot-down of Brothers to the Rescue aircraft.

U.S. Department of Justice. (2026, May 20). United States unseals superseding indictment charging Raúl Castro and five Castro regime co-defendants.

<https://www.justice.gov/opa/pr/united-states-unseals-superseding-indictment-charging-raul-castro-and-five-castro-regime-co>

## **3. SANCTIONS: Financial Landscape**

Note: All embargoes are sanctions, but not all sanctions are embargoes.

### **CACR General Licenses and the Humanitarian Exception**

BLOCKED:

- Direct transfers to Cuban banks or individuals
- Personal remittances via U.S. card networks
- Commercial transactions with Cuban entities
- Payments routed through OFAC-blocked Cuban financial institutions (e.g. Banco Central de Cuba)

PERMITTED:

- Donations to U.S.-registered NGOs and humanitarian organizations operating in Cuba
- Medical, disaster relief, and food aid projects (CACR §515.575)
- Transactions that go to a licensed U.S. entity — not to Cuba directly

- Food donations to NGOs/individuals are explicitly exempt from CACR prohibitions (§515.206(b))

Source: Office of Foreign Assets Control. (n.d.). Cuba sanctions FAQs #740, #741 | CACR §515.206(b), §515.575. U.S. Department of the Treasury.  
<https://ofac.treasury.gov/faqs/topic/1541>

### **How Remittances Reach Cuba**

Formal channels: banks, Western Union, remittance agencies, parcel agencies, postal offices, digital platforms (e.g. TropiPay).

Informal channels: trusted travelers, personal travel, friends, hawala-style brokers, cryptocurrencies.

Usage among Miami-based senders (survey data):

- 69% use formal channels: parcel agencies (28%), Western Union (22%), remittance agencies (15%)
- 63% use informal channels: personal travel (44%), "mules" (38%)
- 48% use both formal and informal channels

### **Why a Foreign Visa Card Works in Havana**

OFAC explicitly permits this:

- CACR §515.584(c): Visa/Mastercard may process card transactions for third-country nationals in Cuba
- OFAC FAQ #740: U.S. card network operators may process and settle transactions for foreign (non-U.S.) financial institutions in Cuba
- OFAC FAQ #741: This applies even to U.S. banks' foreign branches

The jurisdiction boundary: U.S. sanctions apply to "U.S. persons" — citizens, residents, U.S.-incorporated entities. A German bank issuing a Visa card is not a U.S. person — it follows German/EU law, not OFAC. Visa's SEC filing notes that "limited activities may occur involving Visa-branded cards issued outside Cuba by non-US institutions."

Source: Office of Foreign Assets Control. (n.d.). FAQs #740, #741 | CACR §515.584(c) | EU Blocking Statute Reg. 2271/96 | Visa SEC Filing (EDGAR 2014). U.S. Department of the Treasury. <https://ofac.treasury.gov/faqs/topic/1541>

### **The EU Blocking Statute**

Regulation 2271/96 (updated 2018) prohibits EU companies from complying with certain U.S. extraterritorial sanctions — originally created specifically to counter U.S. Cuba policy. It requires EU firms to report to the European Commission if U.S. sanctions affect them, and allows recovery of damages caused by U.S. sanctions compliance.

Practical limit: EU companies still mostly comply with U.S. sanctions — losing U.S. market access vastly outweighs EU fines. The statute is more of a political shield than an enforcement tool.

Council Regulation (EC) No 2271/96 of 22 November 1996 protecting against the effects of the extra-territorial application of legislation adopted by a third country. (1996). *Official Journal of the European Communities*, L 309, 1–6.

<https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX%3A31996R2271>

### **U.S. vs. EU Enforcement**

U.S. position: If you deal with sanctioned Cuban sectors, you lose access to the U.S. market. Enforcement is aggressive — billions in fines, loss of banking licenses. Outcome: most global companies "over-comply" with the U.S. to avoid risk.

EU position: It is illegal for EU companies to follow these U.S. rules. Enforcement is historically weak — companies are rarely punished for "business decisions." Outcome: the EU often files a complaint with the WTO but cannot stop the U.S. Treasury.

### **Sanctioned Country vs. Sanctioned Person**

Country-level sanctions (e.g. Cuba): broad regulatory framework targeting an entire nation, with built-in carve-outs — general licenses, humanitarian exceptions, third-country carve-outs. Administered via the CACR (Cuba) and ITSR (Iran) — detailed rulebooks with permitted categories. Goal is economic pressure on a government, not full financial isolation.

Individual SDN designation: binary — on the list, frozen out. No licenses, no exceptions. All U.S.-jurisdiction assets blocked immediately; no U.S. person may transact with you. Secondary sanctions threat: foreign companies risk U.S. market access if they deal with SDN-listed individuals. ICC judge designations (2020/2025): controversial use of this instrument against international legal officials.

Why Visa, Mastercard, and banks always choose U.S. compliance over EU obligations: OFAC fines and loss of dollar-clearing access are existential. EU Blocking Statute fines are minor and rarely enforced. Over-compliance is common: banks err toward blocking ambiguous transactions rather than risk OFAC scrutiny.

## **4. MEDICAL & PHARMACEUTICAL SUPPLY CHAIN**

### **Cuba's Pharmaceutical System**

- FARMACUBA (under BioCubaFarma): manages the import of raw materials, packaging materials, pharmaceuticals, reagents, equipment, and spare parts for Cuba's pharmaceutical and biotechnological industry. Produces roughly 60–80% of domestic needs.
- MediCuba (under MINSAP): distributes both locally produced and imported medicines throughout Cuba. Imports approximately 38% of drugs not made locally.

- ENCOMED: manages markets, establishes quantities, makes payments, and coordinates the movement of goods and their entry into Cuba for drugs not produced domestically.

### **How U.S. Sanctions Create Obstacles**

1. The USD/banking chokepoint. Any transaction that touches the U.S. dollar — even between two non-U.S. parties — requires clearance through a U.S. financial institution. U.S. banks are prohibited from processing Cuba-related transactions.
2. The chilling effect on willing suppliers. Overcompliance. In 2019, the airline Emirates rejected a shipment contracted by MediCuba from an Indian manufacturer, claiming it could not transport goods destined for Cuba.
3. The 10% U.S.-content rule. A rule from the U.S. Department of Commerce prohibits the export of goods to Cuba by companies in third countries if those goods contain even 10% de minimis U.S.-origin content.

These constraints are further documented in the 1997 American Association for World Health report:

- "A ban on subsidiary trade has severely constrained Cuba's ability to import medicines and medical supplies from third country sources. Moreover, recent corporate buyouts and mergers between major U.S. and European pharmaceutical companies have further reduced the number of companies permitted to do business with Cuba."
- "Numerous licenses for medical equipment and medicines have been denied on the grounds that these exports 'would be detrimental to U.S. foreign policy interests.'"
- "Since 1992, the embargo has prohibited ships from loading or unloading cargo in U.S. ports for 180 days after delivering cargo to Cuba."
- "Though information materials have been exempt from the U.S. trade embargo since 1988, the AAWH study concludes that in practice very little such information goes into Cuba or comes out of the island due to travel restrictions, currency regulations and shipping difficulties. Scientists and citizens of both countries suffer as a result" (AAWH, 1997)

American Association for World Health. (1997). Denial of food and medicine: The impact of the U.S. embargo on health and nutrition in Cuba.

<https://www.american.edu/centers/latin-american-latino-studies/upload/impact-of-us-embargo-on-health-nutrition-in-cuba-1997.pdf>

Oxfam America & Washington Office on Latin America. (1997). Myths and facts about the U.S. embargo on medicine and medical supplies.

[https://www.wola.org/sites/default/files/downloadable/Cuba/past/cuba\\_myths\\_facts.pdf](https://www.wola.org/sites/default/files/downloadable/Cuba/past/cuba_myths_facts.pdf)

### **Major Pharmaceutical Suppliers**

India: #1 supplier. Top source of finished drugs. \$32 million in pharma exports in FY24.



Mexico: #2 supplier. Largest by volume: 82% of tracked shipments. Low freight cost.

Vietnam: #3 supplier. Growing generics source. Trade often handled in local currencies.

How these suppliers bypass U.S. restrictions:

- Non-USD payment rails: Cuba has been actively building alternatives, integrating into Russia's MIR payment card system and China's UnionPay.
- Structuring deals through non-U.S.-exposed suppliers: India and Mexico source finished generics manufactured with no U.S. content, keeping them below the 10% threshold. Smaller Indian generic manufacturers have less U.S. market exposure and are therefore less afraid of OFAC risk than large multinationals.
- State-to-state aid framing: A significant portion of India's pharmaceutical shipments to Cuba are framed as humanitarian or government-to-government transfers, which reduces the commercial paper trail that attracts bank scrutiny.

## 5. THE CUBAN HEALTHCARE SYSTEM

### Structure: Four Tiers

Cuba built a successful four-tier healthcare system under difficult circumstances that rivals those of the wealthiest nations.

- Tier 1 — The consultorio: community-based clinic staffed by a doctor-nurse dyad who live in the community they serve. No computers — patient files are recorded by hand and delivered daily to the polyclinic. The basic health team also includes an OB/GYN, pediatrician, psychiatrist, social worker, and statistician from the regional polyclinic.
- Tier 2 — Polyclinics: approximately 450 across the country. Specialists visit the consultorios from here.
- Tier 3 — Hospitals
- Tier 4 — Complex institutes, handling specialized care and research

From there, the doctor can push patients up the ladder based on complexity and specialist needs, while being present for the patient. This is an example of continuity of care that is well managed.

"One of the things I suspect might be part of the secret sauce that we are missing in the U.S. is this role of community. The doctor is a part of the community and is able to think about the health of her patients as the people she lives with, and literally and metaphorically bumps elbows with every single day."

Seervai, S. (Host). (2019, June 28). Cuba: Where primary care is all about community [Audio podcast episode]. In *The Dose*. Commonwealth Fund. <https://doi.org/10.26099/z6r9-6020>

## Key Health Outcomes

Cuba dedicated a high proportion of GDP to healthcare, with thousands of medical professionals in a total of fifty-six countries, even as its domestic system struggled from workforce shortages and wage-driven brain drain into the hospitality sector.

In 2017, Cuba's infant mortality rate was half that of the city of Washington, D.C. However, as of 2025, Cuba's infant mortality rate has gone up to 9.9 out of 1000 births. Main et al. (2026) note that "recent studies show a strong causal relationship between sanctions imposed and increased death rates," citing a Lancet Global Health study of how sanctions cause approximately 564,000 deaths annually.

Main, A., Sammut, J., Weisbrot, M., & Long, G. (2026, April 27). *US sanctions and the sharp rise in infant mortality in Cuba*. Center for Economic and Policy Research.  
<https://cepr.net/publications/us-sanctions-and-the-sharp-rise-in-infant-mortality-in-cuba/>

Yaffe, H. (2026, March 23). US attacks on Cuban medical missions risk damaging healthcare for poor people in developing countries. *The Conversation*.  
<https://theconversation.com/us-attacks-on-cuban-medical-missions-risk-damaging-healthcare-for-poor-people-in-developing-countries-278748>

"The impact of the crisis caused the Cuban gross domestic product (GDP) to decline by 34.8 percent from 1990 to 1993. Imports decreased by 75 percent, the annual fiscal deficit rose to 33 percent, and the money supply increased to 66 percent of GDP, yielding enormous inflationary pressures. Left on its own, the Cuban economy entered one of its darkest moments, known as the "Special Period in Time of Peace" (Yordanov, 2023).

Radoslav Yordanov (2023). The Long Misunderstanding: Cuba's Economic Ties with the Soviet Bloc. *Journal of Cold War Studies* 2023. 25 (4): 24–52. doi:  
[https://doi.org/10.1162/jcws\\_a\\_01169](https://doi.org/10.1162/jcws_a_01169)

## Cuban Medical Internationalism and Its Economic Dimension

The four forms of Cuban medical internationalism established in the 1960s are:

- Emergency medical brigades overseas
- Treatment of foreign patients in Cuba
- Training foreign students as healthcare professionals
- Establishment of public healthcare facilities overseas

Even with tens of thousands of medical workers overseas, the state's investment in healthcare and medical training means the Cuban population has the highest ratio of doctors per person in the world.

"The oil-for-doctors program between Cuba and Venezuela, formalized in 2004, transformed the export of healthcare professionals into Cuba's greatest revenue source. The U.S. government's

attack on Cuban medical internationalism began in 2006 — the year after that program took hold” (Yaffe, 2026).

Since 2025, the Trump administration has imposed visa restrictions on government officials in Brazil, Grenada, and several African countries for working with Cuban medical programs (Toosi & Bazail-Eimil, 2026).

The Economist. (2020, April 4). Cuba's doctors are in high demand. *The Economist*.  
<https://www.economist.com/the-americas/2020/04/04/cubas-doctors-are-in-high-demand>

Toosi, N., & Bazail-Eimil, E. (2026, February 28). Memo lays out Trump's squeeze on Cuban doctor program. *Politico*.  
<https://www.yahoo.com/news/articles/memo-lays-trump-squeeze-cuban-163000883.html>

Yaffe, H. (2026, March 23). US attacks on Cuban medical missions risk damaging healthcare for poor people in developing countries. *The Conversation*.  
<https://theconversation.com/us-attacks-on-cuban-medical-missions-risk-damaging-healthcare-for-poor-people-in-developing-countries-278748>

## **Testimonials:**

This section documents firsthand accounts from individuals with direct experience of Cuba's healthcare crisis, from inside the system and from the diaspora. Each entry has been anonymized, contributors are identified by role and affiliation only.

These testimonials are primary sources. They reflect personal experience and professional observation, not institutional positions. Readers are encouraged to treat them as qualitative evidence alongside the data and context sections of this repository.

### **Entry 1 - A Cuban Surgeon Speaks:**

**Role:** Practicing surgeon, Cuba

**Affiliation:** Public hospital system (location withheld)

**Date:** May, 2026

**Format:** Video call (Zoom)

**Language:** English

**Conducted by:** Madelin De Jesus Martinez, University of Chicago Student

**Key themes:** Healthcare infrastructure, daily conditions, coping mechanisms, medical supply shortages, misunderstandings about the crisis

This interview was conducted with a Cuban surgeon currently practicing within Cuba's public hospital system. The interviewee has direct experience managing patient care under conditions of

severe resource scarcity, including shortages of medicine, medical equipment, and basic infrastructure.

#### Healthcare system context:

The interviewee noted that no hospitals have been formally closed, but that services within hospitals have been significantly curtailed. The distinction matters: facilities remain open, but the care available inside them has contracted.

#### What are the biggest day-to-day challenges physicians currently face?

The interviewee described the daily experience of practicing medicine under current conditions as one of compounding hardship across every dimension of life — commute, clinical environment, and home.

With no functioning public transport, the interviewee described biking 8 kilometers to the hospital at 7am in 30-degree Celsius heat as part of the normal working day. On arrival, physicians may find shortages of beds, saline, and antibiotics. Because some hospital services have closed, patients are redirected to facilities that may themselves have no available beds. Power outages mean that even resting at home between shifts is not guaranteed.

“ The worst is that it's hell at home, hell at the hospital; it's hell feeling like you are triage, even though war hasn't been declared.”

#### How are physicians coping with scarcity?

Despite the severity of conditions, the interviewee described a culture of collective resilience that operates both inside and outside the hospital.

"Cubans are the only humans that laugh at their misery."

Community functions as the primary coping unit. Neighbors often share food, support, humor, and drinks. Physicians and nurses are deeply valued in Cuban society; the interviewee described a mechanic who refused payment because the patient was a doctor. WhatsApp chat groups keep healthcare workers connected and maintain morale even during blackouts, through jokes and shared information.

Interviewee identified how significant donations have been to keeping services running. International medical students, from the Colombia, Dominican Republic, Panama, Honduras, and Palestine, returned to Cuba with suitcases filled with donated supplies. Cuban physicians working abroad in Nigeria, the Democratic Republic of Congo, Algeria, Qatar, and the United

Arab Emirates send remittances and supplies home. However, the interviewee noted that the number of international students has declined significantly, and that approximately one third of medical residents have applied for leave of absence.

“The way to survive is by creating a community.”

What the world misunderstands:

The interviewee spoke directly to the gap between how the crisis is perceived internationally and how it is experienced on the ground. Cuba has no parallel private healthcare system. There is no alternative infrastructure to fall back on. Healthcare is a constitutional right in Cuba, and the government is its sole provider. When sanctions reduce the government's capacity to import, the health system absorbs that loss directly, there is no buffer.

The interviewee referenced public statements by U.S. officials claiming that sanctions target the Cuban government rather than the Cuban people, and rejected the distinction.

“It’s hard to understand for someone who hasn’t been to Cuba. When you are trying to destroy the Cuban system, you are destroying the health system, because there is no other system.”

The interviewee also noted that under the current pressure, even though things are really hard, the people imposing all the measures have not been able to promote a national disobedience, because the Cuban people know the government is doing their best, with the little they have, and Cubans are not ready to give up.

The interviewee hopes things get better soon, and closed with stating with a direct statement about documentation and responsibility:

**“I hope this suffering is documented; it cannot be forgotten.** [This is the responsibility of the people who are not in Cuba experiencing this or witnessing this firsthand.]”

Cuban Surgeon (anonymized). Personal interview. (2026, May).

Reed, G. (2008, May). Cuba's primary health care revolution: 30 years on. Bull World Health Organ. <https://pmc.ncbi.nlm.nih.gov/articles/PMC2647439/>

**Entry 2 - Conversation with a Cuban-American University of Chicago Administrator**

**Role:** Administrative staff, University of Chicago

**Affiliation:** University of Chicago, Child of a Cuban patient

**Date:** May, 2026

**Format:** In person

**Language:** English and Spanish

**Conducted by:** Madelin De Jesus Martinez, University of Chicago Student

**Key themes:** Diaspora experience, personal family impact, equity and access, contrast between Cuba's past and present healthcare system

Background:

This entry is drawn from a conversation with a member of the University of Chicago's administrative staff who was born and raised in Cuba and later immigrated to the United States. She retains close family ties in Cuba and has direct, ongoing experience navigating the gap between resources available in the U.S. and those accessible to her family on the island. This was an informal conversation.

A family crisis and the cost of inaccessibility:

Her father suffered a heart attack requiring medication that must be administered within 2 hours. Family members drove for hours searching the black market, by the time it was found, the window had closed.

She now constantly sends medications and supplies from the U.S. She grew up with free education and free polyclinic access throughout her formative years in Cuba. That system no longer exists as she knew it.

Her father relies on expensive cardiac medication only available on the black market -- prices are dramatically inflated, and access is both physical and financial.

Living across the divide:

As someone living in the United States, the interviewee described a continuous, practical necessity: she regularly sends medications and supplies from the U.S. to her family in Cuba. This is not an occasional gesture but a sustained responsibility that has become part of ordinary life.

She grew up with free education from primary school through university, and with ready access to polyclinics and free healthcare throughout her formative years in Cuba. The contrast between that experience and the current state of the same system is, in her account, stark and painful.

Equity and the four-tiered reality:

Her father depends on expensive cardiac medication that is now only accessible through the black market. The barrier is twofold.

The first barrier is physical: knowing where to find the medication, and being able to reach it. It is not sold through any formal channel, making time-critical medications near-impossible to secure in a crisis.

The second barrier is financial: black market prices are dramatically inflated, well beyond what most Cuban families can afford without outside support.

**Diaspora Advantage:** The interviewee's situation illustrates a structural inequity that has emerged within an already stressed system. Cubans with relatives abroad, who can send money, medications, or supplies, have a survival advantage that is unavailable to those without connections to people outside Cuba.

**A System Built on Solidarity is Fracturing:** Cuba's healthcare system designed on the principle of universal access. The current crisis is not just a supply shortage, it is eroding the equity principle the system was built on.

Cuban-American University of Chicago Administrator (anonymized). Personal interview. (2026, May)

## **6. POPULATION HEALTH IMPACT**

### **Who Is Affected Inside Cuba**

- Families bringing their own supplies to hospitals
- Patients buying medicine via chat groups — sometimes at 50 times the official cost
- Caregivers to the elderly, often elderly themselves, exhausted and facing scarcity
- More than 1 million of Cuba's ~10 million residents left between 2022 and 2023, fracturing families
- Doctors not paid enough to cover basic living expenses — switching to tourism and hospitality
- Since February 2026: health services reduced to emergency care, mother-child health, and cancer treatment only

*"Those who suffer first and suffer most are ordinary people, especially the most vulnerable."*

— Francisco Pichón, UN Resident Coordinator in Cuba

García, Y. M. (2026, March 3). In Cuba, government mismanagement and US oil moves tell in human suffering. The New Humanitarian.

<https://www.thenewhumanitarian.org/news-feature/2026/03/03/cuba-relentless-us-pressure-human-suffering>

Bellinger, R. (2017, May 8). *Why Cuba's brain drain looks completely different*. University of Maryland Robert H. Smith School of Business.

<https://www.rhsmith.umd.edu/news/why-cubas-brain-drain-looks-completely-different>

United Nations News. (2026, March 26). Cuba blockade hits healthcare.

<https://news.un.org/en/story/2026/03/1167203>

## Who Is Affected Outside Cuba

- Healthcare experts worldwide blocked from Cuba's medical innovations
- Countries that rely on Cuban doctors — especially in rural and underserved areas
- Americans largely unaware of the humanitarian crisis caused by U.S. policy
- Information sharing between Cuban and non-Cuban healthcare professionals restricted
- UN humanitarian plan covering ~2 million people — \$68 million funding gap

American Association for World Health. (1997). Denial of food and medicine.

<https://www.american.edu/centers/latin-american-latino-studies/upload/impact-of-us-embargo-on-health-nutrition-in-cuba-1997.pdf>

Yaffe, H. (2026, March 23). US attacks on Cuban medical missions risk damaging healthcare for poor people in developing countries. *The Conversation*.

<https://theconversation.com/us-attacks-on-cuban-medical-missions-risk-damaging-healthcare-for-poor-people-in-developing-countries-278748>

United Nations News. (2026, March 26). Cuba blockade hits healthcare.

<https://news.un.org/en/story/2026/03/1167203>

## Current Crisis: Key Facts (2026)

"The US decision to block oil shipments to Cuba has added a new strain to a longer erosion of the state's capacity to sustain everyday life, threatening to turn the country's chronic fragility into a full-blown humanitarian collapse. As healthcare, food systems, electricity, and public services deteriorate rapidly and simultaneously, Cubans are left with few avenues for formal support" (Garcia, 2026).

"President Trump's moves in early January — attacking Venezuela, abducting President Nicolás Maduro, and suspending and intercepting all Venezuelan oil shipments to Cuba — deprived the island of its main fuel supplier. The executive order on January 29 also led to a halt in oil imports from Mexico and Russia" (Garcia, 2026).

"Amid rampant inflation and a semi-paralysed economy, President Miguel Díaz-Canel, appointed in 2018 by his predecessor Raúl Castro, acknowledged recently that Cuba's GDP had decreased by 4% in the first nine months of 2025 alone" (Garcia, 2026).

Source: García, Y. M. (2026, March 3). In Cuba, government mismanagement and US oil moves tell in human suffering. *The New Humanitarian*.

<https://www.thenewhumanitarian.org/news-feature/2026/03/03/cuba-relentless-us-pressure-human-suffering>

- No oil imported to the island since January 9, 2026
- Multiple nationwide blackouts — power outages lasting up to 20+ hours a day



- Airlines curtailing flights, devastating the tourism sector
- Hospitals struggling to maintain emergency and intensive care services
- Uncollected garbage accumulating and spreading disease

Agence France-Presse. (2026, March 26). Cuba's escalating health crisis is 'deeply concerning,' says WHO chief. *Le Monde*.

[https://www.lemonde.fr/en/international/article/2026/03/26/cuba-s-escalating-health-crisis-is-deeply-concerning-says-who-chief\\_6751817\\_4.html](https://www.lemonde.fr/en/international/article/2026/03/26/cuba-s-escalating-health-crisis-is-deeply-concerning-says-who-chief_6751817_4.html)

García, Y. M. (2026, March 3). In Cuba, government mismanagement and US oil moves tell in human suffering. *The New Humanitarian*.

<https://www.thenewhumanitarian.org/news-feature/2026/03/03/cuba-relentless-us-pressure-human-suffering>

"Health should be protected at all costs and never be at the mercies of geopolitics, energy blockades and power outages. The situation in Cuba is deeply concerning as the country struggles to maintain health service delivery at a time of immense turbulence."

— WHO Director-General Tedros Adhanom Ghebreyesus

Agence France-Presse. (2026, March 26). Cuba's escalating health crisis is 'deeply concerning,' says WHO chief. *Le Monde*.

[https://www.lemonde.fr/en/international/article/2026/03/26/cuba-s-escalating-health-crisis-is-deeply-concerning-says-who-chief\\_6751817\\_4.html](https://www.lemonde.fr/en/international/article/2026/03/26/cuba-s-escalating-health-crisis-is-deeply-concerning-says-who-chief_6751817_4.html)

### **How the Blockade Affects Population Health: Internal Factors**

Cuba's four-tier healthcare system rivals wealthy nations and the government spends a significant portion of its GDP on healthcare. A pillar of its economy has been sending healthcare workers to 56 countries around the world. Since the crisis, healthcare workers have shifted into the hospitality sector, where they can make more money.

Yaffe, H. (2026, March 23). US attacks on Cuban medical missions risk damaging healthcare for poor people in developing countries. *The Conversation*.

<https://theconversation.com/us-attacks-on-cuban-medical-missions-risk-damaging-healthcare-for-poor-people-in-developing-countries-278748>

Seervai, S. (Host). (2019, June 28). Cuba: Where primary care is all about community [Audio podcast episode]. In *The Dose*. Commonwealth Fund. <https://doi.org/10.26099/z6r9-6020>

American Association for World Health. (1997). Denial of food and medicine.

<https://www.american.edu/centers/latin-american-latino-studies/upload/impact-of-us-embargo-on-health-nutrition-in-cuba-1997.pdf>

Cuba's healthcare crisis shows how it has depended on other countries for oil: the Soviet Union until its collapse, and more recently Venezuela, before the U.S. ousted its leader.

### **How the Blockade Affects Population Health: External Factors**

At the height of the COVID-19 pandemic, Vyair Medical bought ventilator manufacturers IMTmedical and Acutronic, immediately banning all sale of ventilators to Cuba.

Cuba is a test case for how international pressure and sanctions systems fail vulnerable populations. Advocacy matters and speaking intelligently about Cuba signals to bodies like the UN that these policies have consequences.

CubaMinrex. (2020, April 16). *Blockade: U.S. company buys key lung ventilator manufacturers, COVID-19 and suspends sales to Cuba.*

<https://misiones.cubaminrex.cu/en/articulo/blockade-us-company-buys-key-lung-ventilator-manufacturers-covid-19-and-suspends-sales-cuba>

Rodríguez, R. (2020, December 18). *U.S. economic sanctions on Cuba in the context of the pandemic COVID-19. Ethics & International Affairs.* Carnegie Council for Ethics in International Affairs.

<https://www.ethicsandinternationalaffairs.org/online-exclusives/u-s-economic-sanctions-on-cuba-in-the-context-of-the-pandemic-covid-19>

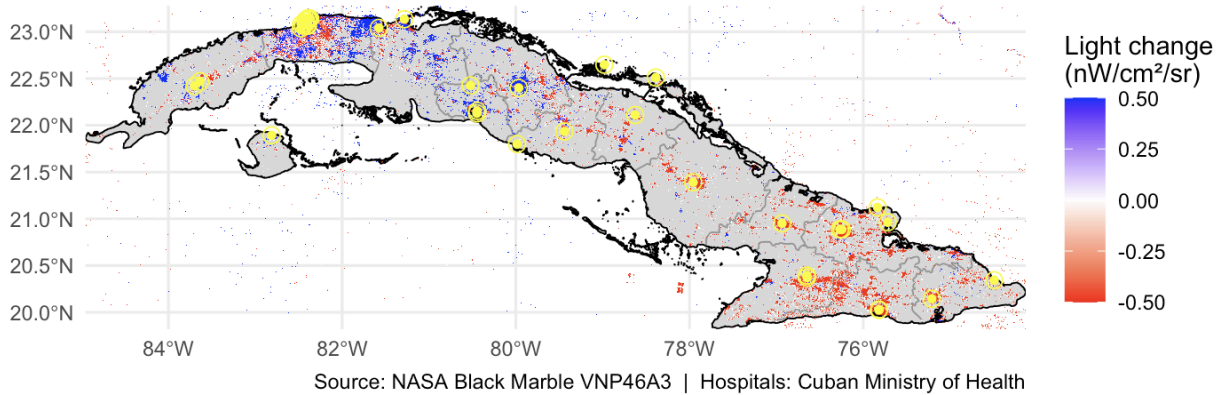
The blockade is making the material conditions of Cubans as difficult as possible for Cubans, directly harming population health outcomes.

### **Visual of non exhaustive list of medical facilities in Cuba, and compares surrounding nighttime light intensity**

Before and after the nationwide electrical collapse and April 2016 (Obama normalization era baseline) vs. April 2026 (current crisis period) using NASA Black Marble satellite imagery.

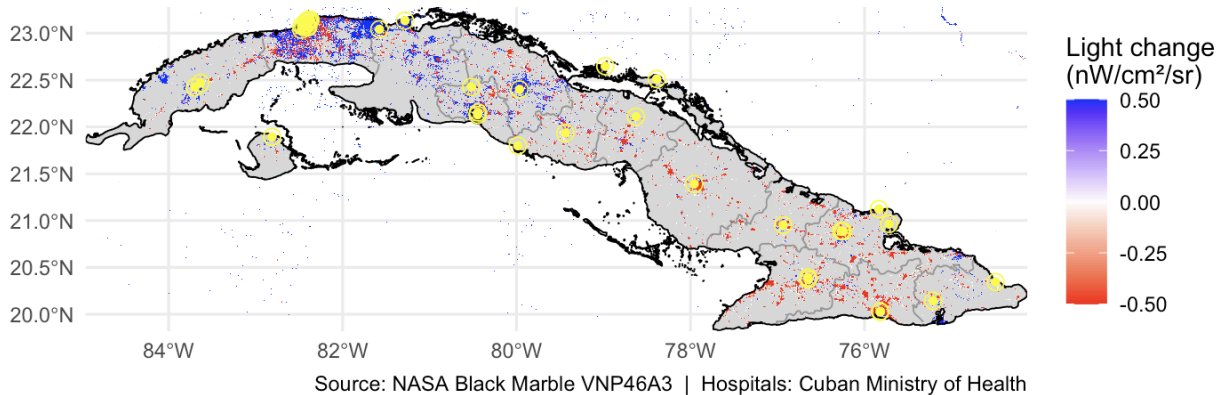
### Cuba Night Light Change: April 2026 vs September 2024

Red = darker (blackout worsening) | Blue = brighter (recovery)  
Yellow circles = 10 km hospital buffers



### Cuba Night Light Change: April 2026 vs April 2016

Decade comparison: Trump-era crisis vs Obama normalization baseline  
Red = darker (long-run deterioration) | Blue = brighter



### Informal Solidarity Networks

Cubans have a long history of informal solidarity networks that fill critical gaps left by the state. Red de Ayuda Humanitaria has raised around €2,000 since October 2025 and imported over 400 kilos of medicines from Spain.

"More than 300 volunteers outside and inside Cuba contribute in different ways, from fundraising and logistics to outreach and coordination."

García, Y. M. (2026, March 3). In Cuba, government mismanagement and US oil moves tell in human suffering. The New Humanitarian.  
<https://www.thenewhumanitarian.org/news-feature/2026/03/03/cuba-relentless-us-pressure-human-suffering>

### **Evidence Base: Sanctions and Health Outcomes**

"Economic sanctions restrict customary commercial and financial ties between states to induce change in political constitution or conduct of the targeted country. Although the stated goals of sanctions often include humanitarian objectives, prospective procedures for health risk assessment are not regularly incorporated in their implementation. Moreover, past experience suggests that the burden of economic isolation may fall on the civilian population" (Pinna Pintor et al., 2023).

Pinna Pintor, M., Suhrcke, M., & Hamelmann, C. (2023). The impact of economic sanctions on health and health systems in low-income and middle-income countries: A systematic review and narrative synthesis. *BMJ Global Health*, 8(2), e010968.  
<https://doi.org/10.1136/bmjgh-2022-010968>

### **Lessons from Venezuela: Sanctions and Public Health**

"The economic crisis in Venezuela has eroded the country's health-care infrastructure and threatened the public health of its people. From 2012 to 2016, infant deaths increased by 63% and maternal mortality more than doubled. Since 2016, outbreaks of the vaccine-preventable diseases measles and diphtheria have spread throughout the region. From 2016 to 2017, Venezuela had the largest rate of increase of malaria in the world, and in 2015, tuberculosis rates were the highest in the country in 40 years" (Page et al., 2019).

Page, K. R., Doocy, S., Reyna Ganteaume, F., Castro, J. S., Spiegel, P., & Beyrer, C. (2019). Venezuela's public health crisis: a regional emergency. *Lancet*, 393(10177), 1254–1260.  
[https://doi.org/10.1016/S0140-6736\(19\)30344-7](https://doi.org/10.1016/S0140-6736(19)30344-7)

"Public health practitioners have a long tradition of resoundingly condemning the collective punishment of populations, the violation of international law, and the premature death and suffering that result from punitive economic measures" (Zakrison & Muntaner, 2019).

Zakrison, T. L., & Muntaner, C. (2019). *US sanctions in Venezuela: Help, hindrance, or violation of human rights?* *The Lancet*, 393(10191), 2586–2587.  
[https://doi.org/10.1016/S0140-6736\(19\)31397-2](https://doi.org/10.1016/S0140-6736(19)31397-2)

### **Russian Tanker Exception**

Beyond the impact in Cuba, Trump's decision to allow a Russian tanker to break the Cuba blockade for "humanitarian reasons" continues a pattern of allowing Russia to operate with virtual impunity on the world stage. Moscow's testing of an American blockade within its own hemisphere coincides with its intelligence sharing efforts on behalf of Iran amid its ongoing war with the U.S. and Israel.

Seligman, L. (2026, March 30). White House: Russian tanker allowed to break Cuba blockade for "humanitarian reasons." Politico.

## 7. CUBAN MEDICAL INNOVATIONS: What the World Misses

*"Cuban scientists are known for ingenuity in the face of adversity."*

Stone, R. (2026, March 30). As U.S. blockade bites, Cuba's health care and science suffer. *Science*.

<https://www.science.org/content/article/u-s-blockade-bites-cuba-s-health-care-and-science-suffer>

"The idea that Cuba, with only 11 million people, and limited income, could be a biotech power, might be incomprehensible for someone working at Pfizer, but for Cuba it is possible."

— John Kirk, Professor Emeritus of Latin American Studies, Dalhousie University

Augustin, E. (2022, January 5). Cuba's vaccine success story. *The Guardian*.

<https://www.theguardian.com/world/2022/jan/05/cuba-coronavirus-covid-vaccines-success-story>

### Treatments Blocked from U.S. Patients and Researchers

- CIMAVax-EGF: lung cancer vaccine. U.S. access only via Roswell Park clinical trials.
- Meningitis B vaccine: not approved by FDA.
- Interferon Alfa-2b (Heberon® Alpha R): produced in Cuba, blocked.
- Streptokinase: clot-dissolving drug, blocked.
- Teravac-HIV-1: AIDS vaccine in clinical trials in Cuba, blocked.
- 5 COVID vaccines: Soberana 1 & 2, Soberana Plus, Abdala, Mambisa
- Nimotuzumab: monoclonal antibody therapy for cancer.
- HeberFERON: skin cancer treatment.
- NeuroEPO plus (NeuralCIM®): Alzheimer's treatment shown to slow disease progression.
- Heberprot-P: treatment for diabetic foot ulcers that significantly reduces amputation rates. Not available in the U.S.

Cuba's medical information itself is restricted. Travel bans, currency regulations, and shipping difficulties mean advances in Cuban research rarely reach U.S. scientists or patients.

Sources:

American Association for World Health. (1997). Denial of food and medicine: The impact of the U.S. embargo on health and nutrition in Cuba. [CIMAVax, Meningitis B, Interferon Alfa-2b, Streptokinase, Teravac-HIV-1, medical information restriction]  
<https://www.american.edu/centers/latin-american-latino-studies/upload/impact-of-us-embargo-on-health-nutrition-in-cuba-1997.pdf>

Saavedra, D., & Crombet, T. (2017). CIMAvax-EGF: A new therapeutic vaccine for advanced non-small cell lung cancer patients. *Frontiers in Immunology*, 8, 269. [CIMAVax-EGF] <https://doi.org/10.3389/fimmu.2017.00269>

Crombet Ramos, T., Mestre Fernández, B., Mazorra Herrera, Z., & Iznaga Escobar, N. E. (2020). Nimotuzumab for patients with inoperable cancer of the head and neck. *Frontiers in Oncology*, 10, 817. [Nimotuzumab] <https://doi.org/10.3389/fonc.2020.00817>

Bello-Rivero, I., Garcia-Vega, Y., Duncan-Roberts, Y., Vazquez-Blomquist, D., Santana-Milian, H., Besada-Perez, V., & Rios-Cabrera, M. (2018). HeberFERON, a new formulation of IFNs with improved pharmacodynamics: Perspective for cancer treatment. *Seminars in Oncology*, 45(1–2), 27–33. [HeberFERON] <https://doi.org/10.1053/j.seminoncol.2018.04.007>

Sosa Pérez, S., Urrutia Amable, N., Viada González, C. E., Lorenzo-Luaces, P., Sánchez Valdés, L., Crombet Ramos, T., León Monzón, K., Rodríguez Obaya, T. J., & Pérez Ruiz, L. (2025). NeuroEPO plus (NeuralCIM®) in Alzheimer's disease: Post-trial observational follow-up study. *Journal of Alzheimer's Disease*, 108(3), 1422–1435. [NeuroEPO plus] <https://doi.org/10.1177/13872877251386885>

Berlanga, J., Fernández, J. I., López, E., López, P. A., del Río, A., Valenzuela, C., Baldomero, J., Muzio, V., Raíces, M., Silva, R., Acevedo, B. E., & Herrera, L. (2013). Heberprot-P: A novel product for treating advanced diabetic foot ulcer. *MEDICC Review*, 15(1), 11–15. [Heberprot-P] <https://doi.org/10.37757/MR2013V15.N1.4>

## **8. LESSONS FROM COMPARABLE CONTEXTS: Delivering Care Under Sanctions**

### **Four Practical Approaches**

1. Assessment first. Determine what data exists, what is reliable, where the gaps are.
2. Immunization as an entry point. Practical, engages external actors, builds broader health infrastructure under blockade.
3. DHIS2 for data management. Open source, free, works on any device. Syncs when power returns. Used in comparable contexts.
4. Roundtable governance. Government, UN, MSF, local and international NGOs — each with a seat at the table.

Baatz, R. K., Ekzayez, A., Najib, Y., Alkhalil, M., Salem, M., Alshiekh, M. A., & Patel, P. (2024). Vaccination governance in protracted conflict settings: The case of northwest Syria. *BMC Health Services Research*, 24, 1082. <https://doi.org/10.1186/s12913-024-11413-1>

University of Toronto. (2019, June 19). Once caught up in Trump's travel ban, U of T grad builds health information system for Syrian hospitals. *University of Toronto News*.

<https://www.utoronto.ca/news/once-caught-trump-s-travel-ban-u-t-grad-builds-health-information-system-syrian-hospitals>